SIL PROCUREMENT, SUPPLY CHAIN & INVENTORY

Pharmacy

For pharmacists and pharmacy technicians

User manual · 28 October 2026

Figures, limits and approval chains are the system defaults; your hospital may have changed them.

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Pharmacy (pharmacists and pharmacy technicians)

For pharmacists and pharmacy technicians in the main pharmacy and the outpatient, inpatient, theatre and emergency outlets. Read Getting started first. For stock work in your pharmacy (receiving, transfers, adjustments, stock takes, issues to wards), see Stores. The rules are the same.

	Pharmacist	Pharmacy technician
Receive / enter prescriptions, dispense	✔	✔
Verify prescriptions, **override** clinical alerts	✔	–
Approve P / POM sales at the till, approve stock adjustments and stock takes	✔	–
Manage the interaction list, catalog, recalls and quality holds	✔	–
Controlled and cold-chain items	✔	✔

Your menu: *Pharmacy & billing* (Pharmacy worklist, Ward supply, Encounters, Patients, Drug interactions, Sales & receipts), plus the *Inventory*, *Wards & issues* and *Planning* screens for your outlets, and *Point of sale* if you sell at the counter.

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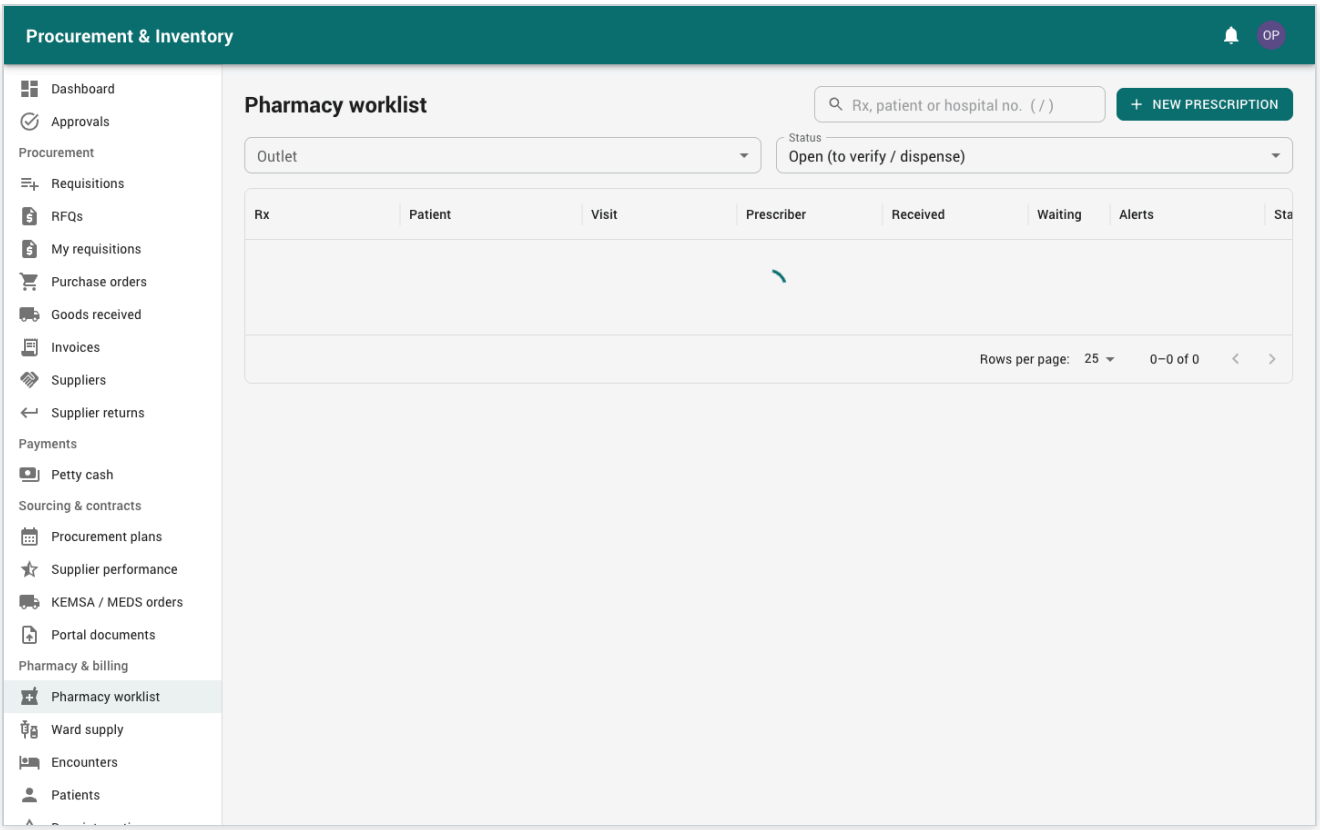
1. Patients (*Pharmacy & billing → Patients*)

The hospital's HMIS is the patient register. This system keeps a copy of what dispensing needs: hospital number, name, date of birth, sex, weight, pregnancy, payer / scheme and **allergies**.

- *Find in HMIS*: search by hospital number or name and import the patient (refreshed automatically when the copy is old).
- *New patient*: only when the HMIS is not connected.
- Keep **allergies**, **weight** and **pregnancy** up to date: the clinical checks use them.

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2. The worklist (*Pharmacy & billing → Pharmacy worklist*)



The pharmacy worklist

Choose your **outlet**. The worklist shows open prescriptions, oldest first, with waiting time and alert badges. It refreshes every 30 seconds.

Prescriptions arrive:

- **from the HMIS** automatically, as doctors order them (mapped drug codes; orders with an unmapped code wait on the HMIS dashboard for the administrator);
- **entered here**: *New prescription* → patient, encounter / visit, prescriber; *Add medicine* at **generic** level with dose, frequency, duration, route and quantity.

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3. Clinical checks

Checks run when a prescription arrives, at verification and at every dispense, against the other lines **and the patient's active medications** (dispensed in the last 30 days):

- drug–drug **interactions** (including each component of combination products);
- **allergies** by substance or class ("penicillins" matches a penicillin antibiotic);
- **duplicate therapy** (same substance: moderate; same class: minor);
- **minimum age** and **pregnancy** warnings.

Severity	What happens
Contraindicated, major	**Blocking**: verification and dispensing stop until a pharmacist overrides the alert
Moderate, minor	Shown in colour; you can continue

Overriding (pharmacists only): *Override clinical alert → Clinical reason (logged and reported)*. Overrides can't be edited or deleted. They are listed under *Drug interactions → Alerts raised vs overridden*. *Re-run checks* after correcting the prescription or the patient's details.

The starter interaction list is marked **DEMO**: it is an illustration, not a clinical reference. Your hospital should import a licensed list (*Drug interactions → Import interactions (CSV)*).

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4. Verify and dispense

1. Open the prescription. Check the clinical checks panel.
2. *Verify* (pharmacist).
3. *Dispense*:
    - choose the **brand** (any brand of the prescribed generic; a substitution is recorded);
    - the **batch** is chosen FEFO; another batch needs the FEFO override and a reason;
    - **partial supply** is allowed. The balance stays open;
    - controlled drugs: a **witness** signs with their username and password.
4. Billing:
    - **billed here** (local billing): the dispense is invoiced. Take payment (cash, M-Pesa, card, insurance / scheme) and print the receipt;
    - **billed in the HMIS**: each item given is charged to the patient's HMIS bill automatically (*Charged to the HMIS bill*); no payment is taken here.
5. The dispense record (batch, expiry, brand) is sent back to the HMIS, so it shows on the patient's record.

Timestamps are kept: received → verified → first dispensed → completed. They feed the turnaround reports.

Cancel undispensed balance (with a reason) closes what will not be supplied.

Returns

Return medicines: choose the dispensed lines and quantities. Returned stock goes *Back into sellable stock (same batch)*. A credit note is raised, or a negative charge on the HMIS bill, and the HMIS record is updated.

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5. Inpatients: ward supply (*Pharmacy & billing → Ward supply*)

Daily and unit-dose orders for admitted patients, by ward and bed. Each line shows what is needed **today** (daily quantity – already given today). Select lines and dispense the round (one dispense per prescription).

Ward stock top-ups are not done here; they are the ward's internal requisitions, issued under *Wards & issues → Issue vouchers*.

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6. Sales receipts and eTIMS

Every invoice and credit note is sent to **KRA eTIMS** through the outlet's control unit. Until eTIMS accepts it, the receipt says **"eTIMS PENDING — THIS IS NOT AN OFFICIAL RECEIPT"**. Once accepted you can reprint the signed receipt (marked COPY) from *Sales & receipts*.

Items without an eTIMS mapping can't be invoiced. Tell the administrator or finance (*KRA eTIMS* console).

Invoices can't be changed after issue; corrections are **credit notes**.

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7. Stock in the pharmacy

- **Receiving**: GRNs with batch and expiry (and fridge temperature for cold-chain items). See Stores, section 3.
- **Transfers** from the main pharmacy to outlets, **adjustments**, **stock takes**: see Stores.
- **Near expiry, reorder, alerts** (*Planning*): see Stores, section 10.
- **Controlled drugs** sit in CD bins; every movement needs a witness.
- **Cold chain**: fridge readings and excursions are under *Quality & safety → Cold chain*. An excursion can quarantine the fridge's batches.

8. Quality and safety (pharmacists)

- **Complaints** (*Quality & safety → Complaints*): report a product quality problem or an adverse drug reaction (PPB report type, reference, date sent); print the report.
- **Recalls** (*Quality & safety → Recalls*): a recall quarantines every affected batch at once. The **trace** lists the stores holding them, wards issued and **patients dispensed (with phone numbers)**, so they can be contacted.
- **Holds**: batches under investigation can't be dispensed or sold until Quality releases them.

9. Selling at the counter

Over-the-counter sales are on the **Till**: see Point of sale. Pharmacists also **approve P / POM sales** for cashiers (your username and password at the till) and can **void and refund**.

10. Reports

Reports → Pharmacy performance: prescriptions and items dispensed (per pharmacist, outlet, day), dispensing turnaround, revenue and margin, top-moving / slow-moving items, ABC and VED analysis, fill rate and stock-outs, alerts overridden, expiry losses.

11. Common messages

Message	Meaning
Blocking clinical alert	Contraindicated / major interaction, allergy or pregnancy flag; a pharmacist must override with a reason
Batch on hold / expired	Pick another batch; Quality manages holds
No stock	Nothing usable at this outlet; request a transfer from the main pharmacy
Item not mapped for eTIMS	The item can't be invoiced until finance / admin maps it
Witness required	Controlled drug: a second, different person with the CD permission signs